

Liberty Medical Center - Gastroenterology Department

789 Elm Street, Suite 201, Springfield, SF 98765

Phone: (555) 987-6543 | Fax: (555) 987-6544

Attending Physician: Dr. Marshall T. Levin, MD

Registration / Demographics

Patient Name:	Lara Dunbar
Date of Birth:	11/05/1980
Age:	43
Sex:	Female
Address:	456 Maple Avenue, Apt. 2A, Springfield, SF 98765
Phone:	(555) 112-3345
MRN:	MRN-67893
Account Number:	ACCT-479005
Insurance:	EverSecure Health Plan - Silver
Emergency Contact:	Mark Dunbar, Phone: (555) 998-1234
Encounter Date:	08/15/2022
Check-In Time:	07:45 AM
Location:	Outpatient Room 5
Attending Gastroenterologist:	Dr. Marshall T. Levin, MD
Referring Provider:	Dr. Janet Roman, MD
Chief Complaint/Indication:	Persistent dyspepsia
Past Medical History:	Hypertension, GERD
Past Surgical History:	Appendectomy
Family History:	Father with peptic ulcer disease
Social History:	Non-smoker, occasional alcohol
Allergies:	Penicillin
Problem List:	Recurrent heartburn, H. pylori infection (resolved)
Current Medications:	

	Omeprazole 40 mg daily, Lisinopril 10 mg daily, Vitamin D 1000 IU daily
Pre-Procedure Vitals:	BP: 132/85, HR: 78, SpO2: 97%, Weight: 150 lbs
ASA Classification:	ASA II

The patient received a thorough informed consent about the procedure, its risks, benefits, and alternatives. Consent was obtained in accordance with standard protocols.

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Pre-procedure Assessment & Anesthesia Record

Pre-Anesthesia Evaluation:	Checked
Airway:	Mallampati Score II, no anticipated difficulties
Cardiovascular:	No murmurs, regular rate and rhythm
Pulmonary:	Clear to auscultation bilaterally
NPO Status:	NPO starting midnight with last intake at 08/14/2022 10:00 PM

Planned Sedation: MAC with propofol 150 mg IV, fentanyl 50 mcg IV.

Anesthesia Care Team

Anesthesiologist: Dr. Steven Gray, MD
CRNA: Alice Kim, CRNA

Procedure Times

Anesthesia Start:	08:00 AM
Procedure Start (Scope In):	08:10 AM
Procedure End (Scope Out):	08:40 AM
Anesthesia End:	08:45 AM
Recovery Start:	08:50 AM
Recovery End:	09:25 AM

Monitoring Trends

Time	BP (mmHg)	HR (bpm)	SpO2 (%)
08:10 AM	128/82	76	100
08:20 AM	130/84	77	98
08:30 AM	125/80	75	99
08:40 AM	120/78	72	97

No anesthesia-related complications encountered during the procedure.

Post-Anesthesia Evaluation

Aldrete Score: 10 - ready for home discharge.

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Procedure Note

Procedure: Esophagogastroduodenoscopy (EGD)

Endoscope: Olympus GIF-2T160; Route: Oral; Position: Left lateral decubitus; Bite Block used.

Under adequate sedation, endoscope was gently introduced into the oropharynx. The upper esophageal sphincter was traversed with ease. Esophageal mucosa was examined carefully noting the Z-line at 38 cm from the incisors. No rings or strictures identified.

The stomach was thoroughly insufflated revealing healthy rugal folds in the body and fundus. Treated mild erythema in antrum with cold biopsy. Pylorus viewed in a relaxed state. Retroflexion in the stomach was unremarkable.

The duodenum was evaluated. No ulceration or abnormal findings in the bulb or second portion. Transparent villi pattern discerning.

Findings:

- Mild erythematous gastropathy - antrum
- Normal appearing duodenum and esophagus

Impression:

Mild gastritis matching symptomatic dyspepsia. GERD to follow up if symptoms persist.

Diagnosis:

Mild gastritis, likely chronic; Note continuation of omeprazole.

Interventions:

Taken two cold forceps biopsies from antrum.

Endoscopy Team: Dr. Marshall T. Levin, MD; Assistant: Daniel Mitchell, CNA; RN Circulator: Ellen Harris, RN; Endoscopy Tech: Joseph Lin

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Specimens / Lab / Pathology

Specimen Collection

Container Label	Site	Fragments	Method	Fixative
A	Antrum	2	Cold forceps	Formalin

Collection Time: 08:30 AM; Sent to Lab at 09:00 AM

Lab Details

Fictitiousville Medical Labs | Accession Number: LAB-TST-452144

Pathology Report

Gross Description: Two pale yellow tan pieces.

Microscopic Description: Biopsies show mild chronic inflammation with no evidence of intestinal metaplasia.

Final Diagnosis: Mild chronic gastritis; Negative for H. pylori.

Pathologist: Dr. Amy Collins, MD

Report Date: 08/17/2022, 11:00 AM

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After Visit Summary / Discharge Instructions

Mrs. Dunbar tolerated the procedure without complications and was monitored post-procedure. She denied any nausea or pain after the recovery.

Diet/Activity Instructions: Resume normal diet as tolerated. Avoid alcohol and NSAIDs.

Medication Plan: Continue with prescribed PPIs. Call physician's office if unexplained symptoms persist.

Results and follow-up consultation will be arranged in four weeks. Transportation today was provided by a family member, who confirmed they understood the discharge instructions.

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Patient was discharged to deemed care and reminded of synergy in the follow-ups.

RN Discharge Educator: Linda Mae, RN
Discharge Time: 09:35 AM

Provider: Dr. Marshall T. Levin, MD

Electronically signed by Dr. Marshall T. Levin, MD on
08/15/2022 at 01:15 PM.